1. Aim: To guide for the admission criteria for ICU

2. **Responsible personnel in the department:**

- ICU Incharge (Intensivist)
- ICU nurse

3. Rationale:

To sustain patient's life in life threatening condition.
Uniform use of critical care admission indicators.

3. Procedure:

✓ Criteria for ICU admission

- The ICU consultant agrees that admission is necessary to provide critical care support that cannot be provided in the ward.

Admitting rights are limited to the ICU consultant/ Primary consultant in consultation with the Intensivist. This is a necessity in that there is a limited number of beds, admission is via a priority system, and the ICU consultant may be the only practitioner informed of every potential ICU admission in the hospital.

- There must be a staffed bed on the unit i.e. appropriate levels of nursing care must be available. Due to safety reasons nurse to patient scoring system is in operation and this should be maintained under all but very

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exceptional circumstances with the consent of medical and senior nursing staff on duty.

- The patient's condition should be potentially reversible. There are circumstances in which emergency treatment already carried out (e.g. Intubation and ventilator) mandate admission to intensive care for at least a brief period even when there is no realistic prospect of survival.

- Admission to the unit should not normally occur for patient with non critical conditions for whom ward care could reasonably be expected.

✓ **Indications for ICU admission**

The following are the indications for admission to the ICU. There may however be other indications for admission to ICU.

- Admission for monitoring:-
  a) Chest pain with ECG changes s/o cardiac disease.
  b) Breathlessness
  c) Palpitations with ECG changes s/o arrhythmia.
  e) Post-operative patients

- Admission for monitoring and treatment
  a) Shock, circulatory failure due to any cause
  b) Respiratory failure
  d) Renal failure, electrolyte disturbances

  e) Any patients requiring invasive monitoring, syringe pump infusions, non-invasive or invasive mechanical ventilation
  f) Septicemia, multisystem organ failure.
  g) Code Blue patient.

- Vital signs
  a) Pulse<60 or >120 beats/minute

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b) Systolic arterial pressure<80 mm of Hg or 20mm of Hg below the patients usual pressure.
c) Mean arterial pressure<60 mm of Hg
d) Diastolic arterial pressure >120 mm of Hg
e) Respiratory rate>35 breaths/minutes.

✓ **Admission decisions:**

Decisions about intensive care admission are clinically based. It is important to understand what constitutes a critical care patient. Most patients fit into one of the following categories:

- Patients requiring support of two or more organ systems, even when this does not include the respiratory system.
- Patients admitted for invasive/intensive in anticipation of possible aggressive interventions.
- Patients with an extensive post operative recovery, allowing abnormal preoperative physiology to reverse, with or without modulation of normal stress response.
- Patients who do not necessarily require life sustaining treatments, but whose physiology is taken under control in order to prevent organ injury e.g. Post anesthesia.
- Patients who have minimal physiological reserve.
- Patients who undergo a major disruption to their physiology, due to an overwhelming stress response to injury, or inadequate compensation to the response e.g. Major trauma or sepsis.

✓ **Standard operating procedure for ICU admission**

- On arrival of a patient, during medical emergency, patient is evaluated in Casualty by the CMO on duty.

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- CMO shall diagnose, stabilize the patient and involve the respective consultant depending on the diagnosis/ type of injury/ procedure/ medical care required.
- After evaluating the patient's condition the CMO/Consultant shall recommend the patient to ICU(If the criteria for ICU admission is fulfilled)
- In case of non availability of bed in ICU, they shall recommend the patient to be transferred to other hospital.
- CMO & the Casualty staff shall hand over the patient & information obtained during evaluation to the ICU registrar/ RMO and staff by means of written notes.
- Proper care shall be taken depending on the physical stability of the patient before and during the transfer.
- The primary consultant/ CMO / Intensivist shall answer the queries of the patient/ attendant regarding proposed care, expected outcomes, expected cost, complications (if any) and other queries of the patient/attendant.
- In the event of MLC, CMO shall enter the details in the medico-legal patients record and intimation of MLC will be sent to the nearest police station.
- In case of unidentified patient brought to the hospital in emergency, the CMO shall call the police and follow the instructions of the police.

✓ **Discharge criteria**:

- Substantial resolution of the problems responsible for admission.
- Anticipation of prolonged medical stability.
- Elimination of need for mechanical ventilation/airway protection.

✓ **Discharge decisions:**

- Discharge decisions are the responsibility of the Consultant on call for the Intensive Care Unit/Primary consultant in consultation with the Intensivist.

- Whenever possible discharges should be anticipated, and arrangements made in good time, with the medical and nursing staff who will be taking over the patient's care.

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- There should be direct verbal contact with a member of the team taking over the patient prior to transfer.

✓ **Protocol for non-availability of bed in ICU / Shortage of bed:**

- A list of 'shift out' and potential shift out' patients will be maintained by the ICU nurse in charge of the shift.
- In case ICU's is full, potential shift out, should be shifted on topmost priority preferably to a room of their choice. If room of their choice is not available, then any available room should be allotted temporarily.
- Such potential shift out should preferably be done during 8am-8pm after appropriate consultation with primary consultant.
- In case of an emergency, shift out may be required from 9pm – 9am. This will be done after appropriate consultation with primary consultant. In case of any disagreement, decision making will rest with the ICU consultant on call.

- In case of non availability of bed in ICU, the patient shall be offered primary care in the casualty after which he shall be recommended to be transferred to other hospitals.

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